Which treatment this guide covers NCCP 00221a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

NCCP 00221 · VERSION 5

Afatinib monotherapy

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

Check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment	Confirm indication and prescribed dose	Daily afatinib; no rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree the next treatment or follow-up
Blood tests	Blood count, kidney and liver profile	Monthly tests; extra checks if unwell	Continue monthly tests	Investigate abnormal results or dehydration	Follow unresolved abnormalities
Heart and eyes	Heart-pumping test if indicated; eye history	Report eye symptoms promptly	Heart tests if clinically indicated	Cardiac/ophthalmology review if symptoms	Agree follow-up for unresolved problems
Symptoms and medicines	Check symptoms, medicines and contact plan	Report early bowel/skin symptoms	Review adherence, supply and interactions	Confirm recovery before any restart	Continue advice for unresolved symptoms
Cancer assessment	Confirm mutation, stage and previous treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

THE WHOLE TREATMENT JOURNEY

Afatinib monotherapy: treatment and checks

EGFR-mutated advanced NSCLC · EGFR-TKI-naïve

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment NCCP 00221 · SMPC	Confirm indication and prescribed dose	Daily afatinib; no rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree the next treatment or follow-up
Blood tests NCCP 00221 · SMPC	Blood count, kidney and liver profile	Monthly tests; extra checks if unwell	Continue monthly tests	Investigate abnormal results or dehydration	Follow unresolved abnormalities
Heart and eyes NCCP 00221 · SMPC	Heart-pumping test if indicated; eye history	Report eye symptoms promptly	Heart tests if clinically indicated	Cardiac/ophthalmology review if symptoms	Agree follow-up for unresolved problems
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Cancer assessment NCCP 00221 · SMPC	Confirm mutation, stage and previous treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

What each check is for

1 Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

Ask / act: Document exact indication, dose and initial investigations.

Sources: NCCP 00221

2 Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

Ask / act: Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

Sources: NCCP 00221 · SMPC

3 New respiratory symptoms

At any time

Possible ILD requires urgent investigation.

Ask / act: Arrange urgent assessment and an authorised medicine plan.

Sources: NCCP 00221 · SMPC

4 Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

Ask / act: Agree continuation, hold, restart, change or follow-up.

Sources: NCCP 00221

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending